

CLINICAL INTELLIGENCE TREND REPORT

YELLOW: Closer Observation Suggested

Patient ID: WimberleyPeriod: December 7 to February 6, 2024Report Date: February 7, 2024Coverage: 1100/1460h (75%)

Patient's monitoring period was less than 90 days; this report covers all available data. Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm
Very High HR > 110 bpm

Low HR < 45 bpm
Elevated Breathing > 24 brpm

Elevated HR > 95 bpm
High Breathing > 30 brpm

High HR > 100 bpm
Very High Breathing > 40 brpm

Patient clinical status over 62 days from Dec 07 to Feb 06

HR Breathing

Elevated Breathing Dec 09 to Dec 11 (3d)	High Breathing Dec 09 to Dec 17 (9d)	High Breathing Dec 27 to Dec 30 (4d)	High Breathing Jan 28 to Feb 01 (5d)	Very High Breathing Dec 07 to Dec 14 (8d)	Very High Breathing Dec 28 to Dec 31 (4d)	Very High Breathing Jan 26 to Feb 01 (7d)
---	---	---	---	--	--	--

Last 24 Hours

Last 24 Hours: NORMAL

30-Day Tier: YELLOW

Vital	Avg	Min	Max
Heart Rate	60 bpm	49 bpm	81 bpm
Breathing	15 brpm	6 brpm	56 brpm

Limited data in the final 24h (16/24h); values reflect available readings.

Last 24 hours: one elevated breathing episode in the morning, otherwise within normal range.

Episodic Events (last 24h)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Feb 06	9 AM to 10 AM	1h	1	Elevated Breathing	26	16	56	Check SpO2 and lung sounds; assess for fluid overload or early infection

81 episodic events Detected, spanning 3 days total. Conditions: Elevated Breathing, High Breathing, Very High Breathing. Priority grade: Medium

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
81	88h	High Breathing	1	75%

Major Findings: Sustained high breathing (avg 34, peak 60)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Jan 26	3 AM to 4 AM	12h	12	Very High Breathing	50	8	60	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Dec 07	9 PM to 10 PM	11h	10	Very High Breathing	49	8	60	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Dec 28	4 PM to 5 PM	4h	4	Very High Breathing	48	14	50	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Jan 28	6 AM to 7 AM	10h	9	High Breathing	33	7	57	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Dec 09	6 PM to 8 PM	17h	14	High Breathing	34	3	50	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Dec 27	2 PM to 3 PM	9h	9	High Breathing	35	9	51	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF

+ 1 additional condition(s) across Elevated Breathing; details in monitoring data.

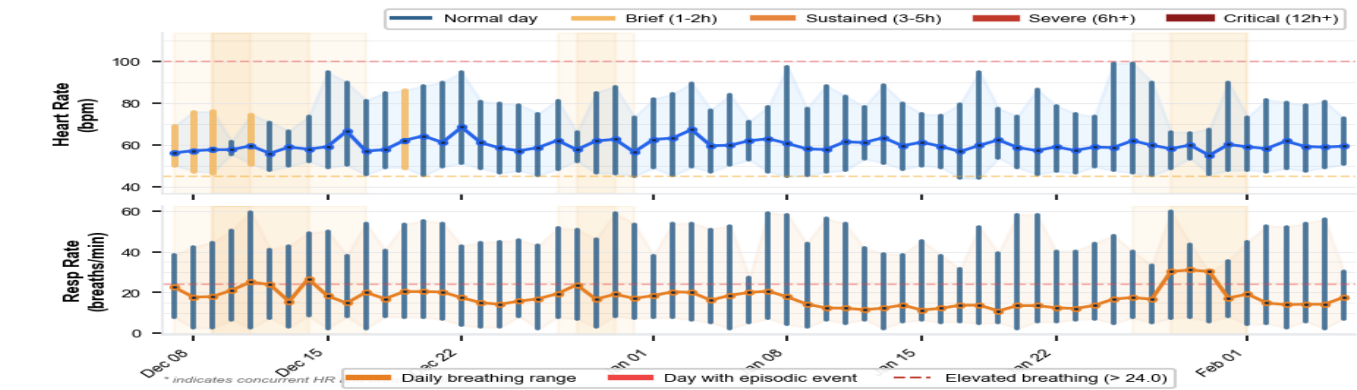
RR values outside physiologic range (6–60 breaths/min).

Suggested Clinical Review Actions

- High Breathing (avg 34 brpm, peak 50 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Very High Breathing (avg 50 brpm, peak 60 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Elevated Breathing (avg 27 brpm, peak 30 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- Overall trajectory shows 7 unstable phases with 17 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

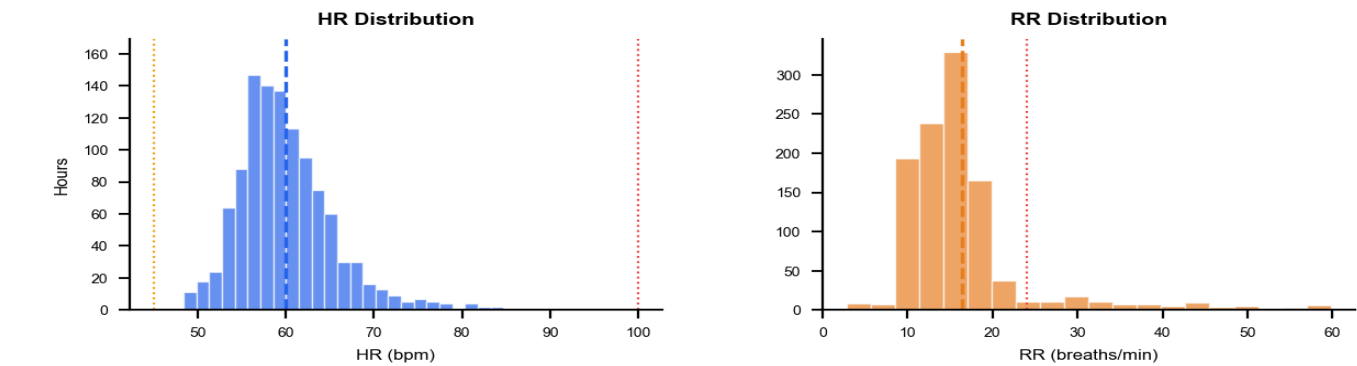
Clinical Pattern Observations:

- 1. Breathing variability: RR spread 21 brpm (10 to 32).
- 2. Clustered pattern: Episodic events on 5 of 62 days (8%).
- 3. Improving trajectory: 34 → 23 events vs prior period.

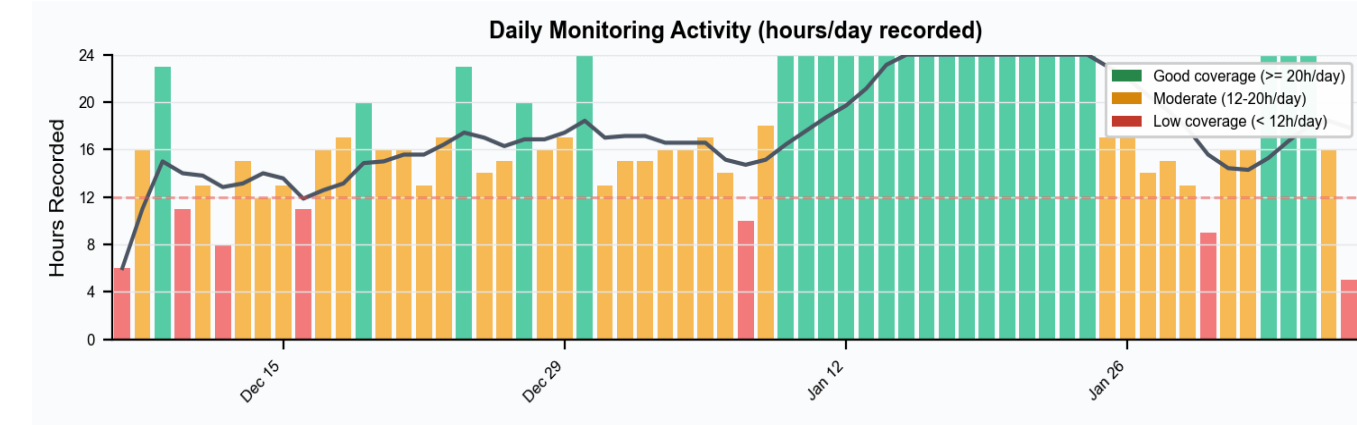


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.